

Is It Lupus? — ANA Stewardship & SLE Workup

Decide whether to test BEFORE you send an ANA. Each feature counts only if unexplained (EULAR/ACR 2019).

Why SLE Looks Like Everything

- UV → apoptosis → macrophage uptake of self-antigen → two pathways:
- **Antibody:** APC → T-helper → B cell → **ANA** → cell destruction, immune-complex deposition, + ANA test.
- **Inflammatory:** antigen → TLR → IL-1, IL-6, IFN-α → inflammation anywhere.
- Autoantibodies brew for years (up to ~9.4 yr) before clinical disease.

Concerning vs “Yellow-Box” by System

System	Concerning → test	Yellow box → don't reflex ANA
Constitutional	Fever ≥38.3°C, unexplained	Fatigue, weight change
Mucocutaneous	Malar/photosensitive rash, alopecia, oral/nasal ulcers, Raynaud's, sicca	Shower hair loss
MSK	Synovitis (red/hot/swollen MCP-PIP-wrist); myositis ↑CK	Aches, pain w/o swelling
Renal	Proteinuria >0.5g, hematuria	Pyuria (infection/STI)
Heme	Cytopenias, hemolysis, VTE	Fe-deficiency anemia
Serosal	Pleural/pericardial effusion, pericarditis	Atypical chest pain
Pulm	ILD, PAH (→ r/o scleroderma)	Dyspnea w/o hypoxia/effusion
GI	AIH, mesenteric vasculitis	Abd pain/diarrhea (IBS), hematochezia (IBD)
GU	>1 or >10-wk miscarriage	Single/early miscarriage
Neuropsych	Psychosis, seizures	Anxiety, depression, brain fog, HA

Diffuse pain + normal exam? Think fibromyalgia / thyroid / OA / spondyloarthritis — not lupus. No red/hot/swollen joint and normal markers argue against SLE.

If Testing Is Indicated — Send the Panel

- **CBC w/ diff** (cytopenias; lympho/neutropenia common) · **CMP** (↑Cr=LN, transaminitis=AIH, ↑bili=hemolysis)
- **C3/C4** (low expected) · **ESR/CRP** (ESR » CRP; high CRP → infection or RA overlap)
- **UA** (hematuria + proteinuria = lupus nephritis; up to 50% of SLE)

Antibody	Meaning
ANA	Must be + to dx; NOT specific
Anti-dsDNA	Specific; tracks activity/nephritis
Anti-Smith	Specific; nephritis
Anti-RNP	MCTD + Raynaud's
Anti-SSA/SSB	Sjögren's, neonatal lupus
Antiphospholipid	30–40% of SLE; clot not required for APS

ANA Stewardship & Pearls

- **Positive ANA in healthy people:** ~32% at 1:40, ~13% at 1:80, ~5% at 1:160 — order for probability, not reassurance.
- **Known SLE + high CRP → infection first** (leading cause of mortality; ESR » CRP in pure flare).
- ≥20% of SLE patients also have fibromyalgia — it explains many overlapping symptoms.
- **Refer rheumatology** when abnormal labs accompany unexplained SLE-specific features; urgent if end-organ damage, severe cytopenia, extensive/bullous rash, or pregnancy.

Treatment (out of scope, 2024 ACR LN): early combination therapy — HCQ for all + steroids + (MPAA+belimumab) or (MPAA+voclosporin) or Euro-Lupus CYC→MPAA+belimumab; taper steroids to ≤5 mg/d by 6 mo. Refer for management.